Sample oral board case

Original case, not from any exam. Answer aloud before reading the examiner’s notes. Give yourself a few minutes with the stem, then turn the page one development at a time.

The stem

A 79-year-old woman is scheduled for hip hemiarthroplasty after a fall. She has severe aortic stenosis (valve area 0.7 cm², mean gradient 46 mmHg on an echo last month), hypertension and mild dementia. She is in sinus rhythm at 76 beats per minute; blood pressure is 152/74.

How to use this

1. Read the stem once. Note what worries you, in order.

2. On page 2, answer each examiner question out loud before reading what the examiner is listening for.

3. Score your answers on the rubric (page 3). Compare with the worked note (page 4).

The examiner's questions

Finding 1

Examiner: How will you induce anesthesia?

What the examiner is listening for:

– A decision first: the technique, then the reasons.

– Arterial line before induction, a slow titrated induction, a vasoconstrictor ready.

– The physiology in one line: fixed outflow obstruction, so protect preload, afterload and sinus rhythm, and avoid tachycardia.

Finding 2

> **Two minutes after induction, blood pressure is 68/40 and heart rate 112.**

Examiner: What do you do, in order?

What the examiner is listening for:

– Treat the pressure now, naming the drug and why it suits a stenotic valve.

– Then the search for a cause: depth of anesthesia, volume, rhythm.

– Why it matters here: a falling diastolic pressure plus tachycardia starves a thick ventricle of coronary flow.

Finding 3

> **The monitor now shows atrial fibrillation at 140. Blood pressure is 62/35.**

Examiner: Your next step?

What the examiner is listening for:

– Recognizing instability, and saying the definitive treatment out loud without hedging.

– Who you call and what you tell the surgeon.

– Whether the case goes ahead once she is stable, and what would change your mind.

Our rubric

Hot Seat teaching rubric. The ABA does not publish its scoring scale; this does not predict an ABA score.

Area	Demonstrated	Partly demonstrated	Needs prompting	Not demonstrated
Judgment	Commits to a safe, specific plan, weighs the tradeoffs out loud, and knows when to stop or cancel	Safe plan with reasonable reasons, though some prompting is needed to commit	Plan is safe but vague, generic, or justified only after probing	Unsafe choice, or can't commit even when pushed
Adaptability	Re-prioritizes immediately when data changes, has a plan B ready, and revisits earlier decisions	Adjusts correctly after new data, with a short lag	Keeps following the original plan until the examiner forces a change	Misses the change or makes it worse
Organization	Answers first, then reasons, in a logical order; concise; specific drugs and monitors	Generally structured, with some rambling or detours	Hard to follow; lists without priorities	Disorganized; the examiner can't tell what they would actually do
Knowledge applied	Explains the mechanism behind each decision, with accurate numbers	Accurate on the essentials, with minor gaps	Gaps that affect decisions, patched when prompted	Factual errors that would harm the patient

Score each area for each finding. When torn between two levels, pick the lower one and write down why.

A worked feedback note

Finding 2, with an illustrative answer. This is the format of the written feedback after a session.

What the candidate said

“Hypotension after induction could be the induction drug, hypovolemia, the valve or an arrhythmia. I'd look at the monitors and think about giving fluid or a pressor.”

Area	This answer	Why
Judgment	Needs prompting	“Fluid or a pressor” leaves the next action undecided.
Adaptability	Partly demonstrated	You include arrhythmia among the possibilities, but do not address the new heart rate.
Organization	Needs prompting	Four possible causes come before an action. State what you will do first.
Knowledge applied	Partly demonstrated	You list possible causes without explaining how they guide your next action.

A more specific opening

“I'm giving a vasoconstrictor now to restore diastolic pressure without speeding her up, then a fluid bolus and lighter anesthesia, and I'm checking the rhythm.”

Next drill: Five stems, out loud, where your first sentence has to be an action.

Want to hear how you sound at exam pace? A 60-minute one-on-one session on Zoom: hotseatanesthesia.com/book